

Procedure Assessment for Resident – Open Cholecystectomy

Resident Resident year ..2 or 3.....OperationOpen Cholecystectomy (OC)...

Date Resident Staff

	F – Fails M – Meets expectation E - Exceeds	F – Fails M – Meets expectation E - Exceeds	Staff comments
Preparation : Short brief an information : Hx, PE, Lab, X-ray	F M E	F M E	
Time out 1. Antibiotics prophylaxis 2. What are the critical or non-routine steps? 3. How long will the case take? 4. What is the anticipated blood loss?	F M E	F M E	
Operation : Incision and approach 1. Kocher's incision 2. Adequate exposure	F M E	F M E	
Relevant anatomy : identify 1. Gallbladder fundus, infundibulum 2. Rouviere's sulcus 3. Calot's triangle 4. Common bile duct	F M E	F M E	
Anticipates steps in procedure 1. Set the retractor for adequate exposure 2. Dissected the hepatocystic triangle and identified cystic duct and cystic artery 3. Clamped and divided cystic duct and cystic artery 4. Dissected GB from GB bed 5. Secured for bleeding and bile leakage 6. Removed GB via plastic bag 7. Abdominal closure	F M E	F M E	
Knowledge of critical points of operation 1. Dissection hepatocystic triangle : cystic duct, cystic artery, cystic plate 2. Management incase of bleeding 3. Management incase of suspected bile duct injury	F M E	F M E	
Demonstrates proper handling of tissue	F M E	F M E	
Good basic skills : suture/knot tying	F M E	F M E	
Follows a logical sequence in case	F M E	F M E	
Responds appropriately to acute events	F M E	F M E	
Follow-Up : Completed Sign-out : Nurse verbally confirms (procedure, instrument, specimen, any problems)	F M E	F M E	
Complete post-operative orders	F M E	F M E	
Saw patient in recovery room	F M E	F M E	

Resident self assessment : What did you learn from this case : _____

Staff assessment : What would you recommend to help the resident improve their skills/ preparation : _____

Overall assessment : F M E

Signature (Staff)

